Cardiac Rehabilitation

The story so far

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Aim

CP+R

The benefits of Cardiac Rehabilitation (CR).

The guidelines set out for CR.

How CR services in the UK are performing compared to guidelines.

CP+R – an overview of our service.

The state of play

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- *Estimated 7 million people living with CVD in the UK.*
- *2.3 million people are living with CHD in the UK.*
- *Premature death, lost productivity, hospital treatment and prescriptions - estimated at £19 billion each year.*
- *175,000 heart attacks in the UK each year; that's one every three minutes.*
- *Incidence of heart failure rising.*

Sourced from British Heart Foundation Cardiovascular Disease Statistics Factsheet[1]

The state of play continued

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+ ***Diabetes** - 3.3 million adults.*
+ ***Smoking –** 1 in 5 adults.*
+ ***Obesity and overweight –** 25% and 33% adults respectively, 33% of children.*
+ ***Physical activity** - 33% of men and nearly 50% of women.*
+ ***Fruit and Veg –** 25% adults, 20% children.*
+ ***Alcohol** - 33% of men and over 25% of women exceed.*
+ ***Blood Pressure –** 25% adults and over half not being treated.*
+ ***Cholesterol –** 50% of adults (5mmol/l or above).*

Sourced from British Heart Foundation Cardiovascular Disease Statistics Factsheet[1]

CR definition

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+ ***The sum of activities required to influence favourably the underlying cause of the disease so that (people) may, by their own efforts preserve or resume when lost, as normal a place in the community…***
+ ***…it must be integrated within secondary prevention services of which it forms one facet.***

The World Health Organisation (WHO). *Cardiac rehabilitation and secondary prevention: long term care for patients with ischaemic heart disease*. Briefing letter. Regional office for Europe: Copenhagen, Denmark; 1993.

Sourced from BACPR Standards and Core Components Document 2012[2]

Evidence to support CR

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+ *Reduction in cardiac mortality (26-36%).*
+ *Reduction in total mortality (13-26%).*
+ *Emerging evidence that cardiac rehabilitation is also associated with a reduction in morbidity.*
+ *28-56% reduction in costly unplanned readmissions.*
+ *Improves functional capacity and perceived quality of life whilst also supporting early return to work and the development of self-management skills.*
+ *One of the most clinically and cost-effective therapeutic interventions in cardiovascular disease management.*

Sourced from BACPR Standards and Core Components Document 2012[2]

Evidence to support CR

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+ *Emphasis now is moving towards early cardiac rehabilitation, commencing within two weeks of either discharge or diagnosis. Starting within this time frame has been shown to be both safe and feasible as well as improving patient uptake and adherence.*

+ *The gains from early cardiac rehabilitation also include the potential to reduce unplanned readmissions early in the discharge period (within 30 days) thus yielding significant cost savings.*

Sourced from BACPR Standards and Core Components Document 2012[2]

The guidelines

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The BACPR (2012) and NICE service guidance (CMG 39 & 40) recommend that:

+ ***CR programmes should be offered early.***

+ ***Underpinned by assessment prior to, and on completion of, CR.***

+ ***The duration and frequency of CR – ideally 12 weeks (or no less than 8 weeks) at a frequency of twice per week.***

+ ***The CR team should be multi-disciplinary with professionals that possess the skills and competences to support patients in achieving the desired health behaviour change.***

Old and New

Traditional Components

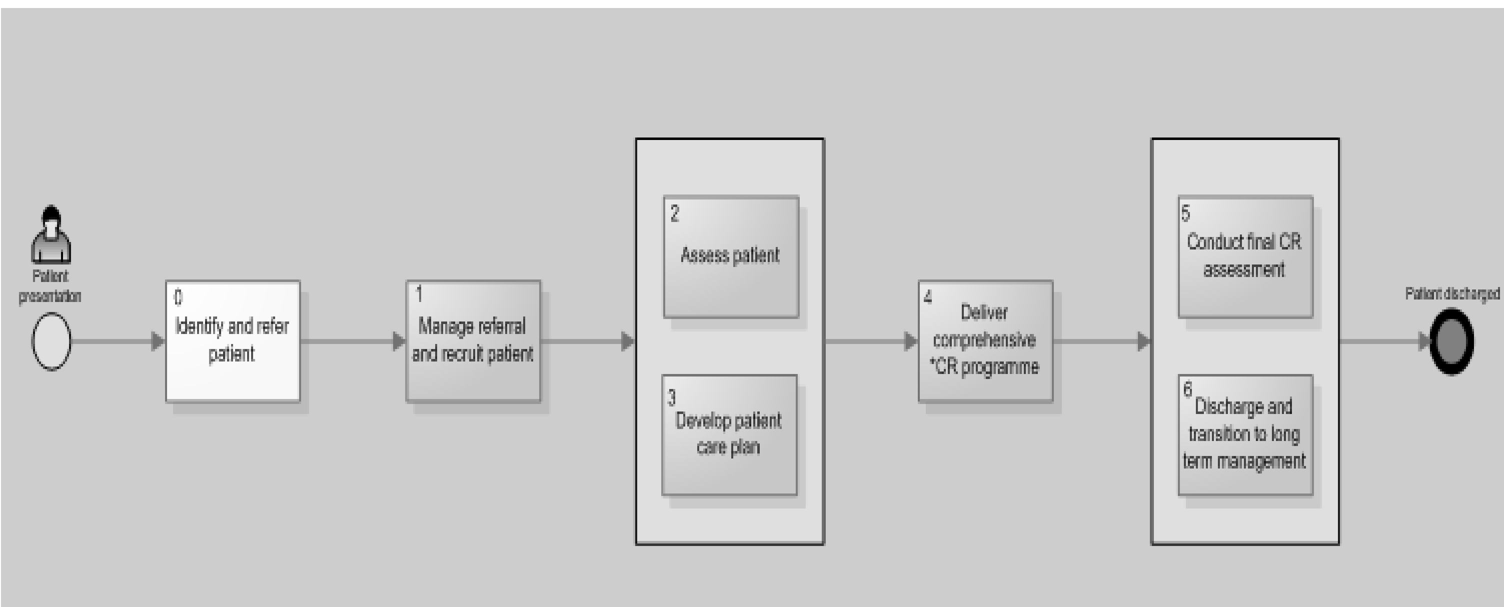
Phase 1
In-patient

Phase 2
Early Post-discharge

Phase 3
CR

Phase 4
Long-term management

Modern Components



The National Audit for CR 2014

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- *Overall uptake to CR in the UK is 45% across CABG, elective PCI, MI and MI+PCI patient groups – NICE goal is 85%.*

- *Uptake to CR ranges from 33% in post MI patients to 82% for patients following CABG surgery.*

- *The recruitment of post MI patients to CR now ranks lowest of the four categories (MI, MI+PCI, PCI (elective), CABG).*

Sourced from The National Audit of Cardiac rehabilitation Annual Statistical Report 2014[3]

The National Audit for CR 2014

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+Significant waiting times remain for cardiac rehabilitation.

+Variation regionally, with uptake ranging from 13% to 88% in different areas.

+Low uptake for some conditions – only 3% of referrals to cardiac rehabilitation from heart failure patients.

Sourced from The National Audit of Cardiac rehabilitation Annual Statistical Report 2014[3]

CR compared to national guidelines

+ *Within 28 days – post MI medical management and PCI.*

2

+ *Within 42 days – post CABG*

0

Sourced from The National Audit of Cardiac rehabilitation Annual Statistical Report 2014[3]

CR compared to national guidelines

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+ *Pre -assessment – inform tailored intervention.*

51%

+ *Post-assessment – evaluate and inform long-term maintenance plan.*

24%

Sourced from The National Audit of Cardiac rehabilitation Annual Statistical Report 2014[3]

CR compared to national guidelines

- *Ideal duration for CR – 12 weeks (84 days) – twice per week.*
- *Minimum duration – 8 weeks (56 days) – twice per week.*

MI - 56

MI + PCI - 54

PCI - 51

CABG - 56

HF - 63

Angina - 53

VS - 56

Other - 56

Sourced from The National Audit of Cardiac rehabilitation Annual Statistical Report 2014[3]

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- *Enhanced CR service*
- *Team-2-one approach*
- *Bridge between hospital and home*
- *Communication*
- *Long-term model*

Clinical Prevention and Rehabilitation

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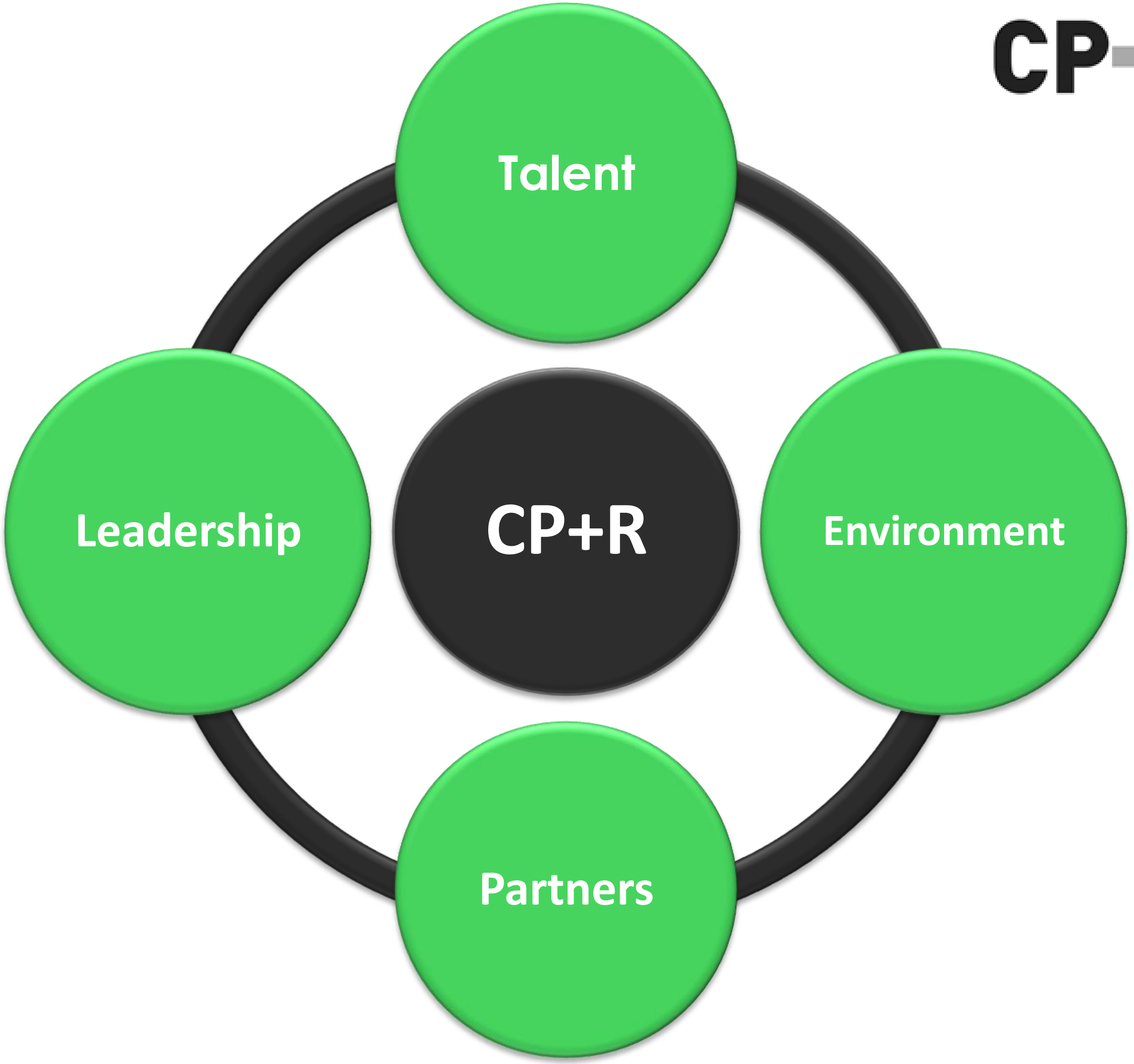
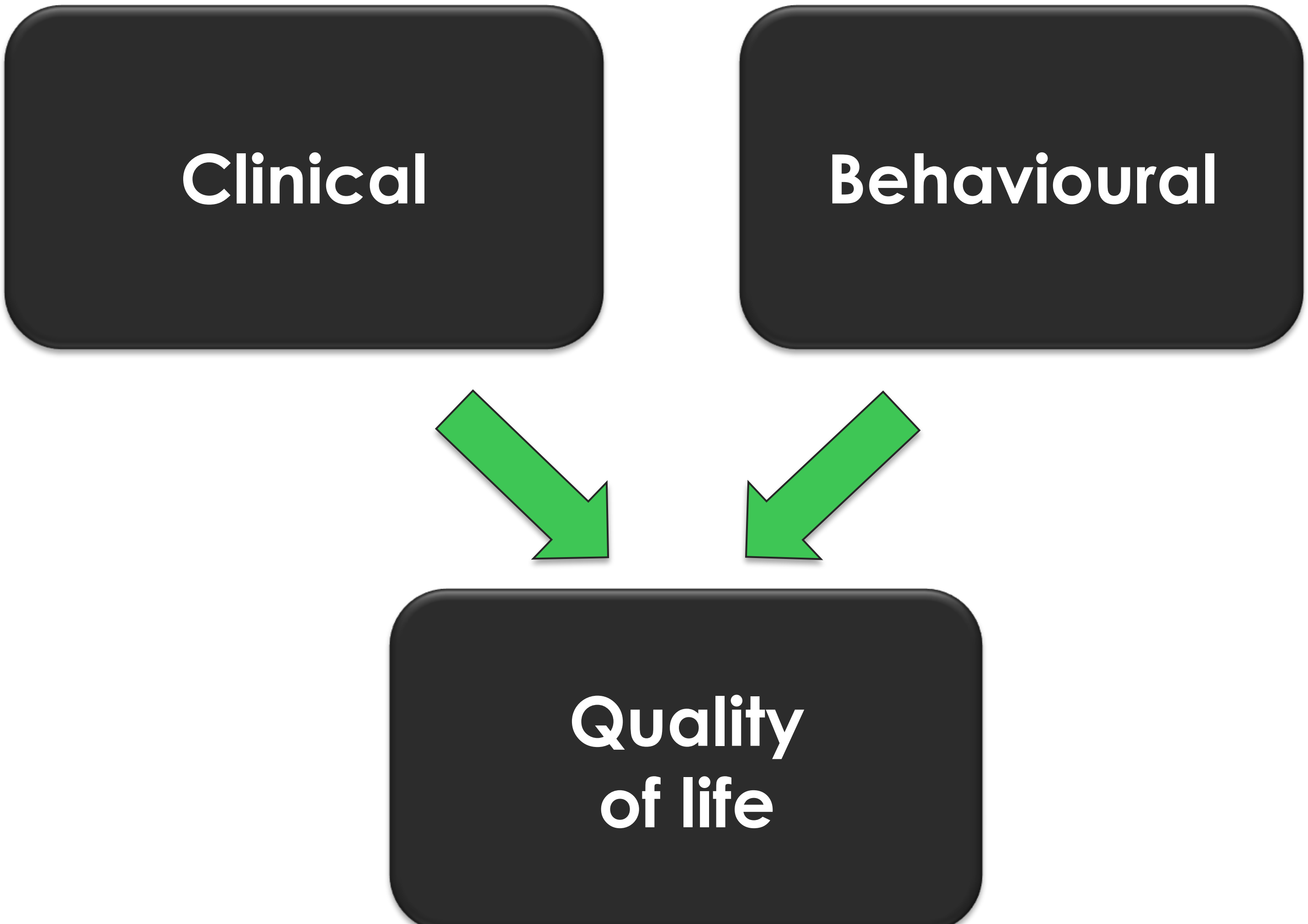


Clinical Prevention and Rehabilitation



Clinical Prevention and Rehabilitation

Well-balanced Lifestyle

Supervised exercise

Minimum of two CP+R prescribed resistance training sessions

Homework

Minimum of two CP+R prescribed cardiovascular training sessions

Nutrition

An 80:20 nutritional practice – using CP+R Nutritional Guidance

Daily movement

Aim for 10,000 steps per day

Thank you

References

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1. *British Heart Foundation – Headline Statistics (last reviewed and update 13th February 2015).*
2. *The BACPR Standards and Core Components for Cardiovascular Prevention and Rehabilitation 2012 (2nd Edition).*
3. *The National Audit for Cardiac Rehabilitation Annual Statistical Report 2014.*